PREOPERATIVE DIAGNOSIS: Infiltrative basal cell carcinoma, left nasal ala.

POSTOPERATIVE DIAGNOSIS: Infiltrative basal cell carcinoma, cleared with Mohs surgery.

PROCEDURE PERFORMED: Mohs micrographic surgery, stage 1 and stage 2, followed by complex linear layered repair of the left nasal ala.

OPERATIVE TECHNIQUE: The patient was brought to the procedure suite. The biopsy site on the left nasal ala was inspected and marked with a 2 mm clinical margin. The surgical site was infiltrated with 1% lidocaine with 1:100,000 epinephrine and 8.4% sodium bicarbonate buffer. The site was prepped and draped in sterile fashion.

Stage 1: Using a #15 scalpel blade, the clinical tumor was excised with a 45-degree beveled incision edge. Hemostasis was achieved with electrodessication. The tissue specimen was mapped, divided into two quadrants, color-coded with red and blue margin dyes, and processed via frozen sections. Microscopic review demonstrated persistent infiltrative basal cell carcinoma at the deep margin of the superior quadrant.

Stage 2: Re-excision of the superior deep margin was performed under local anesthesia. The tissue was mapped, color-coded, and processed. Microscopic examination of stage 2 confirmed complete tumor extirpation with 100% negative peripheral and deep margins.

Defect Reconstruction: The resultant surgical defect measured 14 x 11 mm with depth down to nasal cartilage. A complex layered closure was executed: The deep subcutaneous tissue and muscle were approximated with interrupted 4-0 Vicryl sutures. The cutaneous layer was closed with interrupted 5-0 Prolene sutures with eversion of wound edges. Bacitracin ointment and a pressure dressing were placed.